

Public Health Service Act, 2018

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Chapter-1

Preliminary

1. Short Title and Commencement: (1) This Act may be called the "Public Health Service Act, 2075".

(2) This Act shall come into force immediately. 2. Definitions: Unless the subject or context otherwise requires, in this Act,-

(a) "Emergency Health Service" means the initial and immediate service that must be provided as required to make the life of persons in a life-threatening condition due to an emergency incident or emergency situation safe and to prevent loss of life or limb.

(b) "Basic Health Service" means promotive, preventive, diagnostic, curative, and rehabilitative services that are to be made available easily and free of cost by the State to fulfill the health needs of the general public pursuant to Sub-section (4) of Section 3.

(c) "Non-Governmental or Private Health Institution" means a health institution operating with investment and ownership from the non-governmental or private sector, having obtained approval pursuant to prevailing law.

(d) "Medical Practitioner" means a person registered with the relevant council as a medical practitioner pursuant to prevailing law.

(e) "Public Health Inspector" means the official designated for monitoring and regulation related to public health.

(f) "Hazardous Area" means a work area where there is any chemical, biological, climate change-induced, psychosocial, physical, material, or geographical condition that, due to the workplace or nature of work, has a negative impact on human health, and this term shall also include the potential risk that may occur to health workers during health treatment.

(g) "Prescribed" or "As Prescribed" means as prescribed or as specified in the rules made under this Act.

(h) "Traditional Treatment Provider" means a person who provides treatment using herbs, animal products, and minerals in a traditional manner.

(i) "Ministry" means the Ministry of the Government of Nepal dealing with the subject of health and population.

(j) "Committee" means the National Public Health Committee pursuant to Section 50.

(k) "Government Health Institution" means a health institution operated by the Federation, Province, and Local Level.

(l) "Community Health Worker" means a health worker, including a Female Community Health Volunteer, who has obtained prescribed qualifications or training in health service subjects from a recognized institution and works in an institution or community.

(m) "Service Recipient" means a person receiving health services at a health institution.

(n) "Service Provider" means a medical practitioner, health worker, or community health worker working in a health institution who has obtained specific qualifications or training in health service subjects from a recognized educational institution.

(o) "Health Worker" means a person registered with the relevant council as a health worker pursuant to

prevailing law.

(p) "Health Service" means preventive, promotive, diagnostic, curative, rehabilitative, and palliative services provided based on modern medicine (Allopathy), Ayurveda, Homeopathy, Unani, Naturopathy, Acupuncture, and Sowa Rigpa (Amchi) systems of medicine.

(q) "Health Institution" means a Government Health Institution, and this term shall also refer to non-governmental or private or cooperative or non-profit community health institutions established pursuant to prevailing law.

Chapter-2

Rights and Duties of Service Recipients and Responsibilities of Health Institutions

3. Access to and Assurance of Health Services: (1) Every citizen shall have the right to receive quality health services in an easy and readily accessible manner.

(2) For the purpose of receiving services pursuant to Sub-section (1), it shall be the duty of every citizen to participate in health-related programs prescribed by prevailing law and as specified by the Government of Nepal from time to time.

(3) No citizen shall be deprived of health services.

(4) Every citizen shall have the right to receive free of cost the following prescribed basic health services under the following headings:-

(a) Immunization services,

(b) Maternal, newborn, and child health services such as Integrated Management of Newborn and Childhood Illnesses, nutrition services, antenatal, delivery, and postnatal services, family planning, abortion, and reproductive health,

(c) Communicable disease-related services,

(d) Non-communicable disease and physical disability-related services,

(e) Mental illness-related services,

(f) Senior citizen health-related services,

(g) General emergency condition services,

(h) Health promotion services,

(i) Ayurveda and other recognized alternative health services,

(j) Other services specified by the Government of Nepal by publishing a notice in the Nepal Gazette.

(5) The Provincial or Local Government may add to the basic health services determined by the Government of Nepal pursuant to Sub-section (4) as per necessity.

(6) The financial burden of the services added pursuant to Sub-section (5) shall be borne by the concerned government.

(7) Detailed descriptions of the services pursuant to Sub-section (4), other arrangements and procedures related to service delivery and management shall be as prescribed by the Ministry.

(8) The Local Level may provide basic health services in partnership with health institutions operated under the Province and the Federation.

4. Emergency Treatment: (1) Every health institution must provide prescribed emergency health services.

(2) It shall be the duty of every health institution and health worker to make available the health services pursuant to Sub-section (1).

(3) Notwithstanding anything contained in Sub-section (2), if emergency health services cannot be made available at a health institution, such health institution must immediately provide the treatment available at that institution and refer the person to another health institution for further treatment.

(4) Expenses incurred during treatment pursuant to this Section shall be borne by the concerned person, their guardian, family member, custodian, or person under guardianship if the concerned person has health insurance, from said insurance, and if there is no health insurance or the amount from health insurance is insufficient.

Provided that, health services falling under basic health services must be provided free of cost by the health institution.

(5) While providing emergency treatment services pursuant to this Section, the concerned service recipient shall be made to follow the procedures to be completed for receiving such services only after the treatment has been started.

(6) Other arrangements related to emergency treatment services shall be as prescribed. 5. Specialist Services: (1) The Government of Nepal must make available prescribed specialized services as necessary based on the nature of the service, geographical condition, and disease prevalence rate.

(2) The Provincial Government must make available specialist services according to the standards specified by federal law.

(3) Other arrangements related to making specialist services available shall be as prescribed. 6. Referral Services: (1) If a health institution is unable to provide treatment to a patient coming for treatment due to lack of infrastructure, equipment, specialist services, or any other appropriate reason, it must provide the services available at its institution and immediately refer such patient to a health institution capable of providing further treatment.

(2) While referring a patient pursuant to Sub-section (1), the health institution must complete the methods and procedures as prescribed.

(3) The Government of Nepal, Provincial Government, and Local Level shall make necessary arrangements to make services effective by establishing a necessary referral system between health institutions providing specialist services and basic health services.

(4) Notwithstanding anything contained elsewhere in this Section, while referring a patient under emergency treatment, the referring health institution must refer the patient along with minimum health-related equipment and available health facilities at its disposal. 7. Providing Quality Health Services: (1) The Government of Nepal shall make arrangements to provide quality health services to every citizen from health institutions.

(2) Based on available resources, the Government of Nepal shall perform the following tasks to provide health services to citizens:-

(a) Determining policies for the protection and promotion of citizens' health,

(b) Determining priorities of health services and providing services in an equitable manner.

(3) To implement this Act, the Federation, Province, and Local Level shall establish health institutions meeting prescribed standards based on necessity and arrange necessary manpower, technology, and equipment in such institutions. 8.

Duties of Service Recipients: The duties of a service recipient receiving services at a health institution or from a health worker shall be as follows:- (a) Being conscious about one's own health, adopting a healthy lifestyle, doing yoga and physical exercise, and consuming services provided by health institutions, (b) The person receiving health services shall comply with the rules of the health institution, (c) Providing true information related to their health condition to the health institution and assisting health workers when receiving health services, (d) Respecting the dignity and honor of health workers and the health institution, (e) Not engaging in physical, mental, or gender-based violence towards health workers, (f) In case of refusal to undergo recommended partial or complete treatment, mentioning and signing the treatment card or discharge certificate, (g) Subject to Section 6, going to the health institution referred by the health institution and

receiving services. 9. Responsibility to Arrange Treatment: If a family member, guardian, or any person under guardianship falls ill, it shall be the responsibility of the parent, family member, guardian, or person accepting guardianship to take such person to a health institution, arrange treatment, bear treatment expenses, and provide support and facilitation in the treatment process. 10. Information to be Provided to Service Recipient: (1) Every health institution and health worker providing treatment must provide information to the service recipient on the following matters:- (a) The health and treatment condition of each service recipient, Provided that, if there are sufficient grounds to believe that providing information about the service recipient's health condition may be contrary to the interest of the individual or community, there shall be no bar for the health institution to keep such information confidential. (b) The diagnosis of the disease, the natural outcome of the diagnosed disease, and the level and options of services related to treatment, (c) Estimated costs and potential outcomes related to treatment options, (d) The service recipient's right to refuse health services and the potential consequences, risks, and responsibilities arising from refusing services. (2) While providing information to the service recipient pursuant to Sub-section (1), the health institution shall provide it, as far as possible, in a language that the recipient can understand. (3) Notwithstanding anything contained in Sub-sections (1) and (2), if information about the service recipient's health condition cannot be given to the concerned service recipient, it shall be given to an adult member of their family. 11. Informed Consent of Service Recipient Required: (1) Every health institution must obtain informed consent from the service recipient while providing health services.

Explanation: For the purpose of this Section, "Informed Consent" means written or verbal consent given by a legally competent person to receive health services. (2) For the purpose of Sub-section (1), written consent must be obtained for providing prescribed treatment. (3) Notwithstanding anything contained in Sub-section (1) or (2), health services may be provided without the informed consent of the service recipient in the following circumstances:- (a) If the service recipient is not in a condition to give consent and has not given permission or authority to anyone to provide such consent, and consent is given, in order of availability, by their husband or wife, father or mother, grandfather or grandmother, adult son or daughter, brother or sister, or the nearest available relative of the service recipient, (b) If health services are provided without consent as per prevailing law or a court order, (c) If treating a person poses a serious threat to public health, (d) If delaying the provision of health services could result in the death of the concerned patient or cause irreversible damage to their health, (e) If the concerned person has not refused, explicitly or implicitly or by conduct, to receive health services, (f) Other circumstances as prescribed. 12. Equal Treatment Required: (1) While providing treatment under this Act, it shall be the duty of the concerned health worker to treat all service recipients equally and with respect. (2) Notwithstanding anything contained in Sub-section (1), the health institution may prioritize treatment based on the severity of the patient's health. (3) No health institution shall discriminate or cause discrimination against anyone in treatment based on their origin, religion, color, caste/ethnicity, sex, profession, sexual and gender identity, physical or health condition, disability, marital status, pregnancy, ideological belief, or any other similar ground. 13. Professional Conduct: Health institutions and health workers must follow the professional conduct prescribed by the relevant councils. 14.

Maintaining Confidentiality: (1) While providing health services, information including the service recipient's health condition, diagnosis, or treatment received must be kept confidential. (2) Notwithstanding anything contained in Sub-section (1), this Section shall not be deemed to prevent the disclosure of information in the following circumstances:- (a) If the service recipient gives written consent to disclose the information, (b) If information must be disclosed as per a court order or prevailing law, (c) If non-disclosure of information is seen to cause a serious impact on public health. (3) For the purpose of Clause (c) of Sub-section (2), whether or not there is a serious impact on public health shall be as decided by the prescribed official. 15. Discharge

Summary to be Provided: While discharging a patient admitted and undergoing treatment in a health institution, such institution must provide a discharge summary containing prescribed information. 16. Responsibilities of Health Institution: (1) Every health institution must provide health services only through qualified service providers. (2) Every health institution must adopt necessary preventive measures to prevent and control the transmission of any disease. (3) Every health institution must make arrangements to disseminate the following information related to the health services it provides:- (a) Available health services, their nature, estimated time required, and cost, (b) Operating schedule and schedule of medical practitioners or health workers, (c) Procedures for establishing access to health services, (d) Procedures related to complaint management. 17. Informing Local Administration: (1) If anyone brings a person to a health institution for treatment due to an accident or any other reason, such health institution must immediately provide the treatment services available to it to such person, and if the identity of such person cannot be ascertained, the health institution must immediately give information thereof to the concerned local administration. (2) While giving information pursuant to Sub-section (1), the health institution must also provide information about the treated person and other available related information. (3) Upon receipt of information pursuant to Sub-sections (1) and (2), the local administration must search for the family, parent, or guardian of the concerned person and inform them. 18. Right to File Complaint: If further complications arise in the health of a service recipient during the course of treatment, or if the service recipient has any doubt regarding the treatment process, they may file a complaint against the health worker or health institution involved in the treatment at the prescribed body as prescribed. 19.

Power to Impose Conditions and Restrictions: The head of a health institution may, based on the health condition of a health service provider, impose certain conditions and restrictions on the services provided by the health institution, in accordance with prevailing law. 20. Adoption of Necessary Measures: Every health institution shall, subject to prevailing law, adopt necessary measures for the safety of employees and to prevent and control the transmission of disease. Chapter - 3 Health System and Service Management Related 21. Operation and Regulation of Health Institutions: (1) The tiered structure of health institutions and the services provided by such institutions shall be as prescribed by the Ministry. (2) Minimum standards for health institutions and services shall be as determined by the Ministry. (3) Subject to the standards pursuant to Sub-section (2), every health institution must provide services. (4) Health institutions established and operating prior to the commencement of this Act must meet the standards prescribed pursuant to Sub-section (2) within the time specified by the Ministry. (5) For the delivery of health services, the Federation, Province, and Local Level may enter into necessary partnerships with private or non-governmental health institutions. (6) The arrangement of standards, licensing, and regulation of services provided by private and non-governmental, private or cooperative or non-profit or community health institutions may be determined by the Government of Nepal by publishing a notice in the Nepal Gazette. (7) The Federation, concerned Province, or Local Level may grant licenses to operate health institutions according to the standards determined pursuant to Sub-section (6). (8) Arrangements related to granting licenses, renewals, and monitoring, evaluation, and regulation of such health institutions by the Province and Local Level pursuant to Sub-section (7) shall be as per the concerned provincial and local law. (9) Notwithstanding anything contained in Sub-section (8), until provincial or local law is made, the Province and Local Level may grant licenses, renew, monitor, or regulate health institutions under this Act. 22. License Required: (1) Non-governmental or private health institutions must provide health services only after obtaining a license as prescribed. (2) Health institutions operating without a license at the time of commencement of this Act must obtain a license pursuant to Sub-section (1) within the time specified by the Ministry. (3) In the case of traditional treatment services, services must be provided after obtaining approval according to the standards

specified by the Local Level. 23. Automatic Revocation: The license of health institutions failing to meet the standards prescribed pursuant to Section 21 and operating without obtaining a license pursuant to Section 22 shall be deemed automatically revoked. 24.

Providing Health Services from Locations Other Than Health Institutions: (1) The Local Level and Provincial Government may collaborate with educational institutions to provide school health services. (2) The nature and standards of school health services provided pursuant to Sub-section (1) shall be as prescribed by the concerned Province and Local Level. (3) A health institution may operate specialist services through mobile camps with permission from the prescribed body of the concerned Province. (4) Procedures and standards to be met while providing mobile health camps pursuant to Sub-section (3) shall be as prescribed by the Provincial Government. (5) Procedures and standards to be met while providing home-based health services shall be as prescribed by the Local Level. 25. Health Institution Required to Maintain Quality: (1) The task of ensuring the quality of services provided by health institutions shall be as per federal law. (2) The Ministry may formulate and implement or cause to implement standards for health services. (3) The prescribed body must conduct monitoring and evaluation from time to time regarding compliance with standards under this Section. (4) It shall be the duty of every health institution to follow the directives given by the concerned body after monitoring and evaluation pursuant to Sub-section (3). 26. Management and Development of Human Resources: (1) The Government of Nepal shall prepare and implement policies and standards related to the development, distribution, and utilization of human resources in the health sector. (2) While prescribing standards pursuant to Sub-section (1), it must be done based on long-term, medium-term, and immediate needs analysis and mapping regarding the development, management, and utilization of human resources, and projection of manpower, technology, and equipment. 27. Research, Regulation, Quality Measurement, and Monitoring: (1) The Government of Nepal and the prescribed body may conduct regulation, inspection, monitoring, research, and evaluation regarding whether health institutions and service providers are maintaining quality according to prescribed standards. (2) If it appears that public health is affected due to institutions not complying with the prescribed standards based on the results obtained from inspection, research, and monitoring conducted pursuant to Sub-section (1), the body granting approval for the establishment and operation of the health institution may issue orders including the closure of such institutions. (3) The responsibility of closing or causing to close an institution as per the order under Sub-section (2) shall be of the concerned government. (4) Appropriate mechanisms shall be established for research, regulation, quality measurement, monitoring, and evaluation. 28.

Health Finance and Social Security: (1) A separate health fund shall be established from amounts collected through taxes levied to control goods produced and activities affecting public health. (2) The fund pursuant to Sub-section (1) shall consist of amounts as follows:- (a) Amount received from the Government of Nepal, (b) Amount received from the Provincial Government, (c) Amount received from the Local Level, (d) Amount received from non-governmental and private organizations, (e) Amount received from foreign organizations. (3) The amount of the fund established pursuant to Sub-section (1) must be expended in the field of public health. (4) Arrangements related to the operation of the fund established pursuant to Sub-section (1) shall be as prescribed. (5) Arrangements related to health insurance for citizens shall be as per prevailing law. 29. Provisions Related to Medicine Pricing, Prescription, Pharmacy, and Free Medicines: (1) Medical practitioners and health workers must write the generic name when prescribing medicines. (2) Notwithstanding anything contained in Sub-section (1), in the case of quality domestic products, action must be taken pursuant to Sub-section (1) within two years from the date of commencement of this Section. (3) Notwithstanding anything contained in Sub-section (1), there shall be no bar to prescribing patent and new medicines by brand name. (4) Every government hospital must operate its own pharmacy to ensure regular

availability of medicines at affordable prices. (5) Pricing of medicines shall be as prescribed by the Government of Nepal. (6) Every health institution and service provider must follow standard treatment protocols to ensure the appropriate use and promotion of medicines. (7) Medicine sellers must make necessary arrangements for storage, sale, and distribution in a manner that maintains the prescribed quality of medicines. (8) Medicine sellers shall not sell or distribute any medicine without a prescription from a medical practitioner. (9) Other arrangements related to medicines shall be as per prevailing law. 30. Expenses for Foreign Treatment Not Available: Notwithstanding anything contained in prevailing law, if a person requires treatment abroad, the expenses incurred for such treatment shall not be provided by the Government of Nepal. 31. Extended Hospital Services May be Provided: (1) Health institutions may provide extended hospital services as necessary based on available resources and manpower. (2) Health institutions providing services pursuant to Sub-section (1) must provide additional allowances and other facilities to the medical practitioners and health workers providing such services. 32.

Not Permitted to Serve Elsewhere: (1) Medical practitioners, health workers, or employees working in government hospitals and health institutions shall not serve elsewhere during the time specified by the concerned body. (2) If any medical practitioner, health worker, or employee wishes to serve elsewhere during times other than those mentioned in Sub-section (1), they must obtain prior approval from the concerned body. 33. Emergency Health Treatment Fund May be Established: (1) The Ministry and Provincial Government shall establish an Emergency Health Treatment Fund to arrange treatment for poor and indigent persons, persons whose home address is not traceable, families of martyrs who sacrificed their lives during people's movements, armed conflicts, and revolutions, families of disappeared persons, fighters for democracy, conflict victims and displaced persons, persons with disabilities, injured and victims, who are unable to bear the expenses incurred for their treatment. (2) Arrangements related to the operation of the fund pursuant to Sub-section (1) shall be as prescribed. Chapter-4 Use of Blood, Blood Products and Human Organ Transplantation 34. Blood Transfusion Service: (1) Only institutions licensed by the Ministry may provide blood transfusion services in Nepal. (2) An institution licensed pursuant to Sub-section (1) must, subject to the license, establish units in various places to provide blood transfusion services and other related services. (3) An institution providing blood transfusion services pursuant to Sub-section (1) must, while providing blood to a service recipient, provide only safe blood that is ensured to be free from any type of disease infection. (4) Other arrangements related to granting licenses and renewals under this Section shall be as prescribed. 35. Human Organ Transplantation and Use: Arrangements related to human organ transplantation shall be as per prevailing law. 36. Autopsy and Management: (1) A medical practitioner authorized to conduct autopsies pursuant to prevailing law must conduct an autopsy as prescribed. (2) After an autopsy, unclaimed bodies may be utilized for the study of medical science after completing the procedure as prescribed. 37. Arrangement of Ambulance and Hearse: (1) Health institutions as prescribed by the Ministry must provide ambulance and hearse services conforming to prescribed standards. (2) Other arrangements related to the operation of ambulance and hearse services shall be as prescribed. 38. Identification of Causes of Human Death: If a person dies while undergoing treatment at a health institution, an examination shall be conducted as prescribed to identify the cause of death.

Chapter-5 Social, Cultural, and Environmental Determinants for the Protection, Promotion, and Improvement of Public Health 39. Quality of Consumable Goods: (1) The Government of Nepal may, according to federal law, prescribe minimum quality standards to make consumable goods including food grains, meat, and water of quality and clean during their production, storage, sale, and distribution. (2) It shall be the duty of the Province, Local Level, and all concerned to comply with the standards pursuant to Sub-section (1). (3) Other arrangements related to the quality of consumable goods shall be as per prevailing law. 40. Noise and Air

Pollution: (1) To minimize the impact of noise, air, water, and visual pollution on public health, the Government of Nepal may, according to federal law, determine standards related thereto. (2) It shall be the duty of the Province and Local Level, general citizens, industrialists, businesspersons, and organizations to comply with the standards pursuant to Sub-section (1). (3) To minimize the impact of noise, air, water, and visual pollution on public health, the Government of Nepal shall, according to federal law, determine standards related to noise, air, water, and visual pollution. 41. Sanitation and Waste Management: (1) To control the adverse effects that have been or may be caused to human health by environmental pollution and waste, the Government of Nepal may make necessary standards according to prevailing federal law. (2) The Government of Nepal shall make necessary standards for the collection, reuse, treatment, disposal, and regulation of health-related waste. (3) It shall be the duty of the Province and Local Level to comply with the standards pursuant to Sub-sections (1) and (2). (4) Every health institution must segregate and manage non-hazardous and hazardous wastes according to prescribed standards. (5) Every health institution must provide clean drinking water to service providers according to prescribed standards. 42. Public Health-Friendly Housing, Public Transport, and Road Infrastructure and Safety: (1) While constructing residential areas, standards and quality specified by federal law must be maintained in a health-friendly manner. (2) Road infrastructure must be constructed in a manner that reduces road accidents and promotes health. While constructing new roads and upgrading old roads, provision must be made for footpaths, disability-friendly structures, and bicycle lanes at locations prescribed according to prevailing law. (3) While operating public transport, the Government of Nepal may prescribe and implement or cause to implement necessary standards to make it environmentally friendly so that it does not adversely affect public health. 43.

Industry and Urbanization: (1) To identify and prevent adverse effects that may occur on public health and to minimize risks, a public health impact assessment must be conducted before obtaining approval for prescribed industries, businesses, and projects. (2) The standards, mechanisms, and tiered jurisdictions for the public health impact assessment under Sub-section (1) shall be as specified by federal law. (3) If any industry, business, or project adversely affects public health due to improper waste management or any type of radiation, the concerned industrialist, businessperson, or project must make arrangements for health treatment and appropriate compensation for affected persons, including for physical damage, livestock, and other losses. (4) The Federation, Province, and Local Level must make arrangements for promoting public health-friendly urbanization including adequate play and entertainment spaces, parks, pedestrian walkways, and cycling facilities from a health perspective. (5) While determining grants receivable by the Local Level according to prevailing law, the Government of Nepal must develop and include a public health-friendly indicator. 44. Health Safety of Workers Working in Hazardous Areas: (1) For the health safety of workers working in hazardous areas, the concerned employer must adopt safety measures according to prevailing law. (2) Employers of workers or employees working in hazardous areas pursuant to Sub-section (1) must provide health insurance according to prevailing law. (3) If any employee or worker is injured or dies due to the use of tools, equipment, or any other materials at the workplace in hazardous areas pursuant to Sub-section (1), or if any health worker suffers a serious infection or illness in the course of providing services, the concerned employer must make arrangements for health treatment and appropriate compensation. (4) The concerned employer must provide risk allowance as prescribed by the Government of Nepal to health workers, employees, or workers working in hazardous workplaces pursuant to Sub-section (1). 45. Advertising and Publicity Affecting Public Health: (1) No advertising material that adversely affects human health, including alcohol, cigarettes, tobacco, and tobacco products, shall be produced, distributed, or publicized. (2) No advertisement containing false or misleading information shall be produced, distributed, or publicized to attract individuals towards any material or service that affects mental and physical health. 46.

Social and Cultural Superstitions Affecting Public Health: The Federation, Province, and Local Level shall adopt necessary measures to control social and cultural superstitions that adversely affect public health. 47.

Special Social Security and Programs for Target Groups: (1) The Local Level and Provincial Government must make necessary arrangements to implement special social health security programs for women, children, adolescents, Dalits, families of martyrs, and other target groups specified by the Government of Nepal. (2) Other arrangements related to health services, facilities, and procedures provided for target groups pursuant to Sub-section (1) shall be as prescribed. Chapter-6 Emergency Health Services and Management 48. Emergency Health Services and Management: (1) For the immediate delivery of health services in emergency situations, there shall be a Rapid Response Team and Emergency Medical Team as prescribed. (2) The Federation, Province, and Local Level must develop and implement emergency health plans. (3) While developing plans pursuant to Sub-section (2), the Provincial Government and Local Level must do so in accordance with the standards and directives determined by the Government of Nepal according to federal law. (4) The Local Level may declare a public health disaster situation according to prevailing law. Provided that, if a disaster occurs in more than one Local Level, the concerned Province may declare a public health disaster situation, and if a public health disaster situation arises in more than one Province, the Government of Nepal may declare a public health disaster situation as prescribed. (5) Information regarding the declaration of a public health disaster situation must be publicized through public means for all concerned. (6) The duration, area, and other aspects of a public health disaster may be amended, extended, or rescinded based on available data and information. (7) Other arrangements related to emergency health situations shall be as prescribed. 49.

Chapter-7

National Public Health Committee

50. National Public Health Committee: (1) A National Public Health Committee shall be constituted to provide policy advice on incorporating public health issues into the policies and programs of thematic areas to address the broad social determinants of health affecting human health.

(2) The Committee shall be constituted as follows:-

- (a) Minister, Ministry of the Government of Nepal dealing with health subjects - Chairperson
- (b) Minister of State/Assistant Minister, Ministry of the Government of Nepal dealing with health subjects - Vice-Chairperson
- (c) Member, National Planning Commission (Looking after health subjects) - Member
- (d) One person nominated by the Government of Nepal from among the Vice-Chancellors of Health Science Academies - Member
- (e) Secretary, Ministry of the Government of Nepal dealing with industry subjects - Member
- (f) Secretary, Ministry of the Government of Nepal dealing with finance subjects - Member
- (g) Secretary, Ministry of the Government of Nepal dealing with agriculture subjects - Member
- (h) Secretary, Ministry of the Government of Nepal dealing with water supply and sanitation subjects - Member
- (i) Secretary, Ministry of the Government of Nepal dealing with home affairs subjects - Member
- (j) Secretary, Ministry of the Government of Nepal dealing with physical infrastructure and transport subjects - Member
- (k) Secretary, Ministry of the Government of Nepal dealing with women, children, and senior citizens subjects - Member
- (l) Secretary, Ministry of the Government of Nepal dealing with forest and environment subjects - Member

- (m) Secretary, Ministry of the Government of Nepal dealing with education subjects - Member
 - (n) Secretary, Ministry of the Government of Nepal dealing with labour, employment, and social security subjects - Member
 - (o) Secretary, Ministry of the Government of Nepal dealing with health subjects - Member
 - (p) Chief Specialist, Ministry of the Government of Nepal dealing with health subjects - Member
 - (q) One person nominated from among the Chairs of Health-related Councils - Member
 - (r) One person nominated by the Chairperson from among health experts - Member
 - (s) Two persons, including one woman, nominated by the Chairperson from among operators of private health institutions - Member
 - (t) One person nominated by the Chairperson from among office bearers of organizations related to consumer interest protection - Member
 - (u) Chief, Policy, Planning, and Monitoring Division, Ministry of the Government of Nepal dealing with health and population subjects - Member Secretary
- (3) The term of office for nominated members pursuant to Clauses (d), (q), (r), (s), and (t) of Sub-section (2) shall be three years.
- (4) The Committee may invite other experts to its meetings as necessary.
- (5) The Committee shall meet at least twice a year.
- (6) For the daily functioning of the Committee, the Policy, Planning, and Monitoring Division of the Ministry of the Government of Nepal dealing with health and population subjects shall act as the Secretariat.
- (7) The procedure of the Committee shall be as determined by the Committee itself.